

THE HEALTH REFORM PORTFOLIO

Six Pillars, Proven Sequence, Transferable Framework

Built on Vermont as the Teaching Case

Health Transformation Review

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Preface

Why This Book Exists — And Why the Starting Point Cannot Be a Blank Page

American healthcare is not waiting for transformation to begin. It is already transforming — haltingly, unevenly, and under financial pressure that is forcing changes no stakeholder would have chosen voluntarily. Hospitals are closing maternity wards. Rural emergency departments are converting to lower-acuity models or shuttering entirely. Insurance premiums are consuming an ever-larger share of household income. Primary care physicians are leaving independent practice for hospital employment or leaving medicine entirely. The workforce is exhausted. The administrative burden is crushing. And the populations bearing the worst consequences — rural communities, low-income families, aging Americans — are the ones with the least political power to accelerate change.

This book is written in that reality, not above it.

Every proposal in these pages must be understood as constrained by where the American healthcare system actually is — not where a reformer might wish it to be. There is no blank slate. There is no greenfield on which a theoretically optimal system can be designed from scratch. There is instead a \$4.5 trillion system with millions of employees, thousands of facilities, hundreds of entrenched contractual relationships, fifty state regulatory architectures, multiple federal program structures, and decades of accumulated institutional behavior. Transformation, in this context, does not mean replacing the system. It means systematically altering its fundamental incentives, relationships, and capabilities — from within — in a sequence and at a pace that the system's existing constraints make feasible.

Understanding this is the first and most important premise of the six-pillar framework this book presents. It is also the single most common source of reform failure: the gap between what a policy analysis recommends in isolation and what an embedded, historically conditioned system can actually execute. Reforms designed as if the system were a blank canvas produce technically correct recommendations that collapse on contact with operational reality.

The Inheritance That Reform Must Work With

To build on the American healthcare system is to inherit everything it has accumulated. That inheritance includes genuine achievements that no serious transformation agenda would discard: the most sophisticated biomedical research enterprise in the world; near-universal electronic health record adoption; a partially functional insurance exchange architecture; decades of quality measurement work through HEDIS, HCAHPS, and CMS value-based purchasing; and a growing value-based care contracting ecosystem.

That same inheritance also includes structural features that actively resist transformation. The fee-for-service payment model rewards volume over value. The commercial insurance market's pricing architecture depends on dominant hospital systems charging commercial payers two to four times Medicare rates — a pricing structure that makes premiums increasingly unaffordable while being structurally necessary for hospital solvency. The administrative infrastructure supporting fee-for-service billing consumes an estimated 25 to 35 percent of total healthcare spending. Hospital systems, physician practices, insurance companies, and thousands of ancillary organizations have built their entire operations around this inherited architecture. Transformation that ignores this institutional conditioning will generate resistance that has nothing to do with ideology and everything to do with survival.

What the Historical Record Actually Teaches

The United States has been attempting healthcare transformation for forty years. The pattern of failure is consistent enough to be analytically instructive. The voluntary payment reform era produced one clear finding: voluntary reform achieves marginal results among willing participants while leaving the rest of the system unchanged. Of CMMI's fifty-plus experimental models since 2010, the vast majority produced no statistically significant reduction in spending. Most were discontinued. The technology investment era produced a second finding: technology infrastructure without aligned incentives produces tools that no one has a financial reason to use effectively. The clinical quality improvement era produced genuine quality improvements but did not reduce cost growth. The equity improvement era produced detailed documentation of the problem without systematic reduction in health gaps.

The transformation framework this book presents takes all four of these historical lessons seriously. It does not propose a fifth round of voluntary payment reform. It does not propose technology investment without payment alignment. It does not propose equity programs that leave the underlying resource distribution unchanged.

Vermont as the Teaching Case — And Its Limits

Vermont runs through this book as its primary teaching case because it is the most advanced real-world experiment in simultaneous multi-pillar healthcare transformation currently underway in the United States. Acts 167 and 68 create mandatory reference-based pricing, require global hospital budgets, and mandate a statewide strategic plan with measurable outcome commitments. Vermont is also transparent in ways that make analysis possible — the Oliver Wyman statewide analysis, GMCB monthly data, AHS transformation reports, and 14-hospital transformation plans constitute a documentary record no other state can match.

But Vermont's limits must be stated with equal clarity. Vermont has 647,000 residents, an entirely non-profit hospital system, and the Green Mountain Care Board — a mandatory rate-setting institution that took decades to build. What transfers to other states is the framework: the identification of the six pillars, the sequencing logic, and the portfolio management discipline. What requires adaptation is every specific policy choice.

A Portfolio Approach, Not a Plan

This book frames healthcare transformation as a portfolio management problem. A plan assumes a defined end state and a linear path to reaching it. A portfolio accepts instability as its operating condition. It manages a set of interdependent investments with explicit attention to sequencing, dependencies, risk, and the conditions under which the portfolio must be adjusted. The six pillars are the portfolio's asset classes. The sequencing logic is its investment horizon framework. The dependency analysis is its risk management architecture. Vermont is the portfolio's most detailed case study.

How to Read This Book

For healthcare policy professionals: Read the Preface and Introduction in full, then Chapters 1 and 2 to establish the framework and sequencing logic. Appendix A provides the complete Vermont system portrait.

For healthcare executives and administrators: Read the Introduction for the overarching argument, then proceed to the chapters most relevant to your immediate decisions. Return to Chapters 1 and 2 for the integrating logic.

For Vermont practitioners: Begin with Appendix A, then read Chapters 2 and 3 as the framework most directly grounded in Vermont's current situation.

For students and researchers: Read in chapter order. The Key Concepts glossary at the end of each chapter and the source notes constitute the academic apparatus.

Introduction

The Transformation Imperative: Building on What Exists

I. Starting Where the System Actually Is

On September 18, 2024, the Green Mountain Care Board convened a public meeting at which Oliver Wyman presented the findings of a year-long statewide hospital systems analysis commissioned under Act 167 of 2022. Nine of Vermont's fourteen hospitals were reporting operating losses in 2023, with the worst reaching negative 8.9%. If current trends continued, thirteen of the fourteen would be reporting losses by 2028. The cumulative five-year system deficit was projected between \$700 million and \$2.4 billion. Average premiums for silver exchange plans had increased 108% in six years, while median household income had grown 22%.

Vermont is not an outlier. It is a preview. The structural forces producing Vermont's crisis are present in every American state — demographic aging, hospital financial fragility, commercial cross-subsidy unsustainability, voluntary payment reform failure, workforce exhaustion under administrative burden. Vermont faces them earlier and more acutely. Every state will face them.

But the urgency of Vermont's crisis does not alter the fundamental constraint under which any transformation must operate: the system that needs to be transformed is the system that must do the transforming. There is no external authority that will replace Vermont's hospitals with better-designed ones, no federal program that will install a new payment architecture overnight. Transformation must be built from within the existing system, working with its actual capabilities, its real institutional incentives, and its genuine organizational constraints.

II. The System's Starting Conditions

What Must Be Preserved

The American healthcare system's genuine achievements are not incidental to transformation — they are its foundation. Academic medical centers at the frontier of biomedical research. Near-universal EHR adoption. Decades of quality measurement infrastructure. A partially functional exchange architecture. An established primary care infrastructure. A growing value-based care contracting ecosystem with real performance data. Transformation that destroys these assets while building new ones wastes capital, alienates workforce, and creates political opposition that derails reform.

What Prevents Transformation

- The fee-for-service payment model rewards volume, not value. Every primary care visit that prevents a hospitalization generates no revenue from the prevention. Every hospital that succeeds in reducing its avoidable admission rate reduces its own income.
- The commercial cross-subsidy architecture requires dominant hospitals to charge commercial payers 200% to 400% of Medicare rates to cover below-cost Medicare and Medicaid reimbursement — producing premiums that are increasingly unaffordable while being structurally necessary for hospital solvency.
- Voluntary participation structures allow the highest-cost actors to opt out of reform. The ACO experience from 2010 to 2025 is the empirical record of what voluntary payment reform achieves at scale.
- The administrative burden of fee-for-service billing consumes an estimated 25 to 35% of total healthcare spending and absorbs workforce capacity that should go to patient care.
- Technology investment without payment alignment produces digital infrastructure that no one has a financial reason to use effectively.

III. The Sequencing Question

The most consequential analytical question in any transformation agenda is not what to do. The evidence on what needs to happen has been reasonably settled for decades. The consequential question is in what order to do it. The dependency

structure of the six pillars generates a specific, defensible answer. Some investments cannot produce their intended results until enabling investments are already in place. Getting the sequence wrong is not merely inefficient — it produces structural failures that are expensive to reverse and politically difficult to explain.

The Six-Pillar Execution Sequence — Core Logic	
Stage 1 POLICY:	Mandatory architecture and transformation capital must precede all else. The enabling legislation, regulatory authority, and federal agreements that convert aspirational policy into binding obligation are the preconditions for every subsequent investment. Policy also secures the transformation capital — federal grant awards — that funds the technology build.
Stage 2 TECHNOLOGY:	Data infrastructure must be built before financial risk is assumed. Managing global budgets without integrated, timely clinical and claims data is the 'managing blind' failure mode. Technology is the operating system on which every other pillar's management functions depend.
Stage 3 ECONOMICS:	Payment incentives deploy on a system that can already see itself. Reference-based pricing, global budgets, and total cost of care accountability change the financial logic that shapes organizational and clinical behavior. Economics-as-management begins only when data infrastructure is ready.
Stage 4 CLINICAL:	Care redesign on aligned incentives. Primary care transformation, behavioral health integration, care management at scale — the clinical programs where the economic return on the entire investment is realized. Clinical redesign without payment alignment reverts when the incentives revert.
Stage 5 EQUITY:	Systematic audit and calibration. Equity is a constraining dependency through every earlier stage, not a final add-on. Social risk adjustment in payment design (Stage 3) and targeted clinical deployment (Stage 4) are equity functions that precede Stage 5. Stage 5 measures whether earlier commitments produced equitable outcomes.
Stage 6 OPERATIONS:	Closing the administrative cost gap and translating strategy into implemented programs. Vermont's critical-access hospitals spend \$2,730 per discharge on administration against a national benchmark of \$1,427. Operations is last not because it is least important, but because operational redesign requires the payment environment it is optimizing for to already be operational.

IV. The Six Pillars — An Integrated System, Not a Checklist

The six-pillar framework is the analytical architecture that organizes this book. Each pillar represents a domain of necessary investment; the fifteen dependency relationships between them represent the mechanisms through which each pillar's effectiveness depends on the others.

Pillar	Structural Role — What It Produces When Functioning
Policy	The mandatory architecture. Converts aspirational reform into binding requirements. Secures transformation capital.
Technology	The data substrate. Makes population health management, value-based contract execution, equity measurement, and strategic planning analytically feasible.
Economics	The incentive architecture. Determines whether organizations have a financial reason to behave differently.
Clinical	The mechanism of change. Where payment reform becomes patient outcomes — or fails to.
Equity	The cross-cutting constraint and accountability lens. A transformation that improves averages while widening disparities has failed.

Operations	The execution layer. Translates statutory mandates, payment models, data platforms, and clinical programs into organizational reality.
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V. Vermont's Specific Starting Conditions

The Regulatory Asset: The Green Mountain Care Board

Vermont's most important structural advantage is institutional. The Green Mountain Care Board has had authority to set hospital budgets — not merely review them — since fiscal year 2013. This is mandatory rate-setting authority over all fourteen Vermont hospitals. When GMCB sets a budget, hospitals must operate within it. This regulatory architecture is the Policy pillar's foundation. Other states attempting payment reform without a comparable regulatory body are attempting the Economics pillar without the Policy pillar structure that gives payment reform enforcement authority.

The Financial Crisis

Nine of fourteen hospitals reported operating losses in FY2023. Oliver Wyman projects thirteen of fourteen in losses by 2028 absent structural intervention. The cumulative five-year deficit is projected between \$700 million and \$2.4 billion. Critical Access Hospitals face average operating profit of \$938 per adjusted discharge against a \$2,784 national benchmark. UVMHC has an administrative cost per adjusted discharge of \$3,826 against a national benchmark of \$1,427. These are two different problems requiring two different interventions.

The Demographic Constraint

Vermont's over-65 population will reach over 30% by 2040 — up from 21.7% in 2020. The working-age population will decline 13% from 367,000 to approximately 318,000. This demographic shift increases demand for complex, high-cost care while reducing the commercial insurance base that cross-subsidizes below-cost Medicare and Medicaid payments. Every year without structural transformation is a year in which the financial gap widens and the organizational capacity to execute transformation narrows.

VI. What This Book Can and Cannot Do

This book provides the analytical framework for understanding what is happening in American healthcare transformation and why — the six-pillar architecture, the sequencing logic, the failure cascade predictions, and the Vermont evidence. It does not predict whether Vermont's transformation will succeed, substitute for the operational expertise required to implement transformation in a specific organizational context, or resolve the political contests that surround healthcare transformation. What it can do is give every participant in those processes a more rigorous analytical foundation for the positions they hold and the decisions they make.

The Central Argument in Five Sentences
American healthcare transformation is not a blank-canvas design problem. It is a portfolio management problem constrained by a \$4.5 trillion system's institutional history, financial structure, and organizational capabilities.
The six pillars of transformation — Policy, Technology, Economics, Clinical, Equity, and Operations — are interdependent, and transformation fails when any pillar is addressed in isolation.
The order in which the pillars are activated matters as much as the content of the investments: Policy must establish mandatory architecture before anything else; Technology must precede Economics-as-management; Clinical transformation requires aligned payment incentives; Equity must constrain every pillar's design; Operations executes last but plans throughout.

Vermont is the most advanced real-world experiment in simultaneous six-pillar transformation currently underway in the United States — not because Vermont is large, but because it is early, transparent, and operating under financial pressure severe enough to make sequencing discipline mandatory.

The analytical framework Vermont's experience validates is transferable to every state and health system that will face Vermont's conditions on a ten-to-twenty-year lag — which is to say, every state and health system in the United States.

The Six-Pillar Framework

The Health Reform Portfolio — What the Six Pillars Are, How They Relate, and Why All Must Move Together

Chapter 1 establishes the six-pillar framework that organizes this entire book and to which every subsequent chapter returns. It answers three questions: what each pillar is and what function it performs in the transformation system; what it means to think of those six pillars as a portfolio rather than a checklist; and what the reader will find in each pillar's dedicated chapter. The full architecture of dependency relationships between the pillars is developed in Chapter 1A. This chapter provides the foundation that makes Chapter 1A legible.

I. This Is a Portfolio, Not a Plan

A plan assumes a defined end state and a linear path to reaching it. It assumes that the policy environment, the financial context, the technological landscape, and the organizational capabilities of the implementing institutions will remain stable enough that the document written today will accurately describe what happens over the next five years. In healthcare transformation, none of these assumptions hold. Federal policy changes mid-reform. Hospital systems consolidate or fail. Technology evolves faster than governance can follow. Workforce shortages worsen unexpectedly. Political coalitions shift.

A portfolio is different. A portfolio is a set of active investments, managed together, with explicit attention to how they interact, how their risks correlate, what their sequencing dependencies are, and how the whole must be adjusted when any single element underperforms or the environment shifts. Portfolio management does not commit to a single predetermined path. It commits to a direction, a set of foundational investments that must be made regardless of how the environment shifts, and a decision logic for adaptation.

The six-pillar framework is the architecture of that portfolio. Policy, Technology, Economics, Clinical, Equity, and Operations are its six asset classes. The sequencing logic developed in Chapter 2 is its investment horizon framework. The dependency analysis in Chapter 1A is its risk management architecture. Vermont is the portfolio's most detailed case study — what it looks like to manage all six asset classes simultaneously in a real system under real financial pressure, without the luxury of a blank canvas.

What a Portfolio Approach Requires That a Plan Does Not

Explicit acknowledgment of uncertainty. A portfolio manager does not pretend to know exactly how each investment will perform. The portfolio is structured to produce acceptable outcomes across a range of scenarios, not optimal outcomes in the single scenario the planner expects.

Active management of interdependencies. Portfolio assets interact. A technology investment that fails affects the economics investment that depended on it. A policy reversal at the federal level affects every pillar simultaneously. These interactions must be monitored and managed in real time, not assumed away in the planning document.

Explicit sequencing discipline. Not all investments can be made simultaneously with equal effect. Some must precede others because the later investment cannot function without the earlier one already in place. Portfolio management makes sequencing a design constraint, not a scheduling preference.

Tolerance for partial failure. The portfolio approach treats pillar underperformance as an informative signal requiring adjustment, not a catastrophic deviation from a plan that must be concealed or explained away.

A standard of success measured across the whole portfolio. Individual pillar success that does not produce system-level transformation is not success. The standard is population health improvement, affordability, and equity.

II. The Failure That Makes the Framework Necessary

In 2010, the Affordable Care Act created the Center for Medicare and Medicaid Innovation with a mandate to test new payment and delivery models. Over the following fifteen years, CMMI launched more than fifty models. A 2023 evaluation found that the vast majority had not achieved statistically significant reductions in spending or improvements in quality. Most were discontinued.

The pattern of failure across those models is analytically consistent. Models that addressed only the Economics pillar — changing what Medicare paid without addressing how care was delivered — generated financial pressure without behavioral change. Models that addressed only the Clinical pillar produced quality improvements that the payment system then penalized. Models that required Technology pillar investment found that providers lacked the operational capacity to use the tools effectively.

This is the defining failure mode of American healthcare reform: single-pillar thinking. Each isolated intervention runs directly into the constraints created by the pillars it ignored. The result is technically correct recommendations that collapse on contact with a system that was not redesigned to support them.

“The problem is not the list of what needs to change. Every serious framework for the past thirty years has identified essentially the same list. The problem is treating it as a list — a set of independent items to be checked in any order — rather than as a system of interdependent investments to be sequenced, managed, and held accountable together.” — The Health Reform Portfolio

III. The Six Pillars Defined

The six pillars are not equal in their causal priority — Policy and Technology are enabling preconditions that others depend on — but they are equal in their necessity. A transformed healthcare system requires all six to be functional. Remove any one and the others are undermined. The full dependency architecture is developed in Chapter 1A.

Pillar	Definition, Structural Role, and Vermont Application
Policy	The legislative and regulatory environment that creates the mandatory architecture for transformation. Policy establishes what actors must do, what they are prohibited from doing, and what governance structures ensure accountability. It performs two functions no other pillar can perform: it creates the mandatory architecture that prevents high-cost actors from opting out of reform, and it secures the transformation capital that funds the technology build. <i>Vermont application: Acts 167 (2022), 51 (2023), and 68 (2025) are Vermont's Policy pillar reform cascade. The AHEAD Model State Agreement (January 2025) and the \$195M Rural Health Transformation Program award are Policy pillar products. The Green Mountain Care Board's mandatory rate-setting authority is the institutional Policy pillar asset that makes mandatory payment reform operationally executable.</i>
Technology	The data infrastructure, health information exchange, analytical platforms, and clinical technologies that make population health management possible at scale. Technology is the information substrate on which all other pillar management functions depend — you cannot manage what you cannot measure. It must be in place before financial risk is assumed under a value-based payment model. The OneCare Vermont failure — managing global budget accountability without integrated, timely data — is the canonical proof of what happens when Economics precedes Technology in the execution sequence. <i>Vermont application: VHCURES (all-payer claims database), VTIL/VHIE (health information exchange), the AHS-GMCB analytics vendor procured in early 2026, AI scribe and remote patient monitoring grants through the RHT Program, and Vermont's Clinically Integrated Network are Vermont's Technology pillar investments.</i>
Economics	The payment and financial architecture that determines what behaviors the healthcare system rewards. The Economics pillar encompasses payment reform from fee-for-service to value-based and global payment, financial modeling, total cost of care management, and the

	hospital financial sustainability that transformation requires. A critical internal distinction: Economics-as-design (defining the payment architecture) can happen in parallel with the Technology build; Economics-as-management (running global budgets operationally) requires Technology to already be functional. <i>Vermont application: Mandatory reference-based pricing beginning FY2027, global hospital budgets beginning FY2028, the AHEAD Model's total cost of care accountability, and the AHEAD EAST Fund (up to \$150M annually) are Vermont's Economics pillar instruments.</i>
Clinical	The care delivery models, quality measurement frameworks, clinical workforce design, and patient care programs that determine how care is actually delivered. The Clinical pillar is where payment reform becomes — or fails to become — patient outcomes. Clinical redesign without payment alignment reverts when the incentives revert. Clinical transformation comes fourth because it requires aligned payment incentives and data infrastructure to already be in place. <i>Vermont application: The Blueprint for Health (all-payer PCMH program, 5.8:1 documented ROI), the Collaborative Care Model for behavioral health integration, CCBHC expansion (seven certified entities by mid-2026), and the primary care investment strategy under AHEAD are Vermont's Clinical pillar investments.</i>
Equity	The programs, investments, accountability structures, and payment design requirements that ensure transformation benefits reach all populations. Equity is not a soft afterthought — it is an analytical requirement and a constraining dependency on every other pillar's design. A transformation that reduces average costs while widening disparities has failed. Equity must constrain payment design at Stage 3 through social risk adjustment, not merely be measured at Stage 5. <i>Vermont application: Geographic equity across Vermont's 14 Hospital Service Areas, SDOH investment through the AHEAD EAST Fund and Blueprint HRSN universal screening, rural access preservation requirements in RBP and global budget design, and the Health Equity Advisory Commission's role in the Statewide Strategic Plan are Vermont's Equity pillar investments.</i>
Operations	The organizational capacity, management infrastructure, administrative simplification, and implementation discipline that converts strategy into executed programs. The Operations pillar is where transformation plans become real or remain documents. Operations is sixth in the execution sequence not because it is least important, but because operational redesign must be calibrated to the payment environment it is executing within. <i>Vermont application: The 14-hospital transformation planning process (148 operational commitments as of January 2026), the Rural Health Redesign Center technical assistance methodology, the AHS HSA Coordinator model, and the Vermont CIN's shared services architecture are Vermont's Operations pillar investments. Vermont's critical-access hospitals spend \$2,730 per adjusted discharge on administration against a \$1,427 national benchmark.</i>

IV. The Six Diagnostic Questions

Each pillar answers a specific diagnostic question in the transformation logic. A policy intervention that passes the Policy question but fails the Technology, Economics, or Operations questions is not a solution — it is a well-intentioned mistake waiting to be made.

Pillar	Diagnostic Question	What It Tests	Failure Mode When the Answer Is No
Policy	Is it permissible and mandatory?	Whether the legal and regulatory architecture makes the intervention binding, not merely aspirational. Whether high-cost actors can opt out.	Voluntary reform. The highest-cost actors decline to participate. Results limited to willing participants with favorable populations.

Technology	Is it visible?	Whether the data infrastructure exists to measure what the intervention is supposed to manage — attribution, TCOC, risk stratification, equity stratification, quality gaps.	'Managing blind.' Financial accountability is real; management capability is absent. Value-based contracts produce unexpected losses.
Economics	Is it financially rational?	Whether the payment model creates a financial incentive for the behavior the intervention requires — specifically, whether population health management is rewarded rather than penalized.	Clinical redesign that reverts. Programs demonstrate outcomes but cannot capture the financial return because the payment system still rewards volume.
Clinical	Is it effective?	Whether the care delivery models, workforce design, and clinical programs can actually produce the health improvement and utilization reduction the payment model's economics require.	Financial pressure without clinical pathway. Global budgets create incentive to reduce hospitalizations; without clinical programs, the budget produces access restriction.
Equity	Is it just — and designed to be?	Whether the intervention reduces or exacerbates disparities by race, income, and geography — and whether equity requirements are embedded in the design, not reviewed at the end.	Technically successful, ethically failed transformation. Average outcomes improve; disparities widen. Payment design penalizes safety-net providers.
Operations	Is it executable?	Whether the organizational capacity, management infrastructure, and implementation discipline exist to translate strategy into functioning programs.	The document graveyard. Statutory mandates exist on paper. Clinical programs are designed. Data platforms are procured. Nothing is implemented at scale.

V. The Portfolio Structure — How the Pillars Relate

The six pillars have hard structural relationships that constrain how they can be combined and in what order they must be activated. Three types of relationship organize the fifteen directed connections between the pillars:

Relationship Type	What It Means	Critical Example
Enables (critical path)	The upstream pillar's presence is required for the downstream pillar to perform its core function. If the enabling pillar is absent, the downstream pillar's core function is disabled — not merely weakened.	Policy → Economics: mandatory authority enables structural payment reform. Without the Policy pillar's mandatory architecture, the highest-cost actors opt out. Vermont's decade of OneCare experience is the proof case.
Requires	The downstream pillar's management function depends on a specific output the upstream pillar must produce. The downstream pillar can be designed without the upstream output, but it cannot be managed without it.	Technology → Economics: global budget contracts require data infrastructure. A hospital can sign a global budget agreement without analytics capability. It cannot manage its performance under that agreement.

Constrains	The downstream pillar imposes design requirements on the upstream pillar that must be built in from the beginning, not retrofitted afterward.	Equity → Economics: equity constrains payment design. Global budgets set without social risk adjustment systematically penalize hospitals serving high-SDOH populations.
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Why the Dependency Structure Matters for Real Decisions

SEQUENCING: The dependency structure generates a partial ordering of interventions. Some investments cannot produce their expected results until enabling investments are in place. Getting the sequence wrong produces failed interventions that generate political resistance to the subsequent correct interventions.

RISK IDENTIFICATION: The dependency map identifies transformation vulnerabilities precisely. A pillar with many inbound dependencies is the highest-risk gap in the architecture. In Vermont's current architecture, the Technology pillar has the highest inbound dependency count: Economics, Equity, and Clinical all require Technology outputs. A delay in analytics vendor deployment simultaneously impairs three pillar functions.

INVESTMENT PRIORITIZATION: Not all pillar investments have equal leverage. An investment in a pillar with many inbound dependencies has higher system-level leverage than an investment in a pillar required by only one other. The highest-return investment is not necessarily the one whose individual ROI looks largest in isolation.

VI. The Master Reference Table — The Framework at a Glance

This section is the reference that readers return to throughout the book. Every subsequent chapter assumes familiarity with the definitions, diagnostic questions, and structural roles summarized here.

Pillar	Diagnostic Question	Structural Role	What It Produces When Functioning	Vermont Anchor
Policy	Is it permissible and mandatory?	The mandatory architecture. Converts aspirational reform into binding requirements. Secures transformation capital.	Statutory mandates eliminating voluntary opt-out; accountability timelines; federal-state agreements; transformation grant capital.	Acts 167, 51, 68; AHEAD State Agreement; \$195M RHT award; GMCB mandatory rate authority
Technology	Is it visible?	The data substrate. Makes population health management, VBC contract execution, equity measurement, and strategic planning analytically feasible.	Attribution lists; TCOC measurement; demographic risk stratification; care gap identification; real-time population health dashboards.	VHCURES; VITL/VHIE; AHS-GMCB analytics vendor; Vermont CIN; AI scribe grants
Economics	Is it financially rational?	The incentive architecture. Creates the financial logic that makes population health management rational rather than penalized.	Payment models rewarding prevention; global budgets making volume expansion irrational; total cost of care accountability.	Mandatory RBP FY2027; global budgets FY2028-2030; AHEAD TCOC accountability; EAST Fund up to \$150M/year

Clinical	Is it effective?	The mechanism of change. Converts payment incentives into actual health outcomes and utilization reduction.	Primary care transformation preventing hospitalizations; behavioral health integration; care management closing gaps before they become acute episodes.	Blueprint for Health (5.8:1 ROI); CoCM; CCBHC expansion (7 entities 2026); primary care AHEAD; COE regionalization
Equity	Is it just — and designed to be?	The cross-cutting constraint and accountability lens. Applied to every other pillar's design from the outset, not reviewed at the end.	Stratified measurement; payment designs not penalizing safety-net providers; clinical programs reaching disadvantaged populations; social risk adjustment.	HSA-level geographic monitoring; HRSN screening; NE Kingdom focus; REH model; Health Equity Advisory Commission
Operations	Is it executable?	The execution layer. Translates statutory mandates, payment models, data platforms, and clinical programs into organizational reality.	Hospital transformation plans that are implemented; AHS organizational capacity; PMO tracking statutory deadlines; administrative cost reduction.	RHRC methodology; 148 hospital plan initiatives (Jan 2026); Vermont CIN shared services; AHS HSA Coordinators; \$1,303/discharge admin gap

Sources: six-pillar framework documentation; Acts 167 and 68 of 2025; Oliver Wyman Act 167 Community Engagement: Recommendations (August 2024); Vermont AHEAD State Agreement (January 2025); Vermont RHT Program Application (November 2025); AHS Health Care System Transformation Reports (2025); GMCB records.

The Dependency Architecture

All Fifteen Directed Relationships Between the Six Pillars — How They Work, What Breaks When They Are Violated, and Why the Sequence Is Not Optional

Chapter 1A is the analytical companion to Chapter 1. Where Chapter 1 defines the six pillars and introduces the three types of inter-pillar relationship, this chapter develops all fifteen directed dependency relationships in full — their mechanisms, their failure modes, and the Vermont evidence that tests each relationship against real-world experience. Chapter 2 draws on this dependency architecture to derive the execution sequence.

I. How to Read This Chapter

Each of the fifteen dependency relationships is documented using a consistent structure: the upstream pillar and downstream pillar involved; the type of relationship (enables, requires, or constrains); the mechanism by which the upstream pillar affects the downstream pillar's function; the failure mode produced when the dependency is violated; and the Vermont evidence.

Three relationships are designated critical path: Policy → Technology, Policy → Economics, and Technology → Economics. These are the dependencies most consequential to the execution sequence because their violation produces the most catastrophic and least reversible failure modes.

Relationship Type	Definition	Direction of Causality	Failure Mode When Violated
Enables (critical path)	The upstream pillar's presence is required for the downstream pillar to perform its core function. If the enabling pillar is absent, the downstream pillar's core function is disabled — not merely weakened.	Upstream → downstream. The enabling pillar must be in place before the downstream pillar is activated.	Core function disabled. Interventions that look correct in design produce no structural result because the mandatory architecture or capital that would make them operational is absent.
Requires	The downstream pillar's management function depends on a specific output the upstream pillar must produce. The downstream pillar can exist without the upstream output but cannot be managed without it.	Upstream → downstream. The required output must be available before the downstream pillar's management functions are activated.	'Managing blind.' The intervention is real, the accountability is real, but the organization lacks the information to manage performance under it.
Constrains	The downstream pillar imposes design requirements on the upstream pillar that must be built in from the beginning, not retrofitted after deployment.	Downstream → upstream. The constraining relationship runs against the primary direction of authority.	Technically functional, structurally unjust. Payment models systematically penalize safety-net providers. Clinical programs reach the well-served while leaving the underserved behind.

II. Policy Pillar — Outbound Dependencies (4 Relationships)

The Policy pillar has the highest outbound dependency count in the framework: four directed relationships, two of which are critical-path enablers. Without the Policy pillar's mandatory architecture, the other five pillars are either aspirational programs or voluntary experiments.

Dependency 1: Policy → Technology (Enables — Critical Path)

Mechanism: The Policy pillar secures the transformation capital — federal grant awards, federal-state agreements, state appropriations — that funds the Technology build at a scale no individual institution can finance independently.

Failure mode: Without Policy capital, the Technology build is fragmented into hospital-level investments that cannot achieve the statewide interoperability required for population health management. Individual hospitals invest in EHR systems; no one invests in the exchange layer that makes those systems collectively useful.

Vermont evidence: Vermont's \$195 million Rural Health Transformation Program award (December 2025) is the direct product of Act 167's statutory mandate and the AHEAD Model State Agreement. The award funds AI scribe technology, remote patient monitoring, broadband infrastructure, and Vermont's CIN shared services — all Technology pillar investments that would not otherwise be financially feasible.

Dependency 2: Policy → Economics (Enables — Critical Path)

Mechanism: Mandatory payment reform requires a statutory and regulatory architecture that converts voluntary payment model participation into binding obligation. Without this mandatory architecture, the highest-cost actors rationally decline to participate in payment models that hold them accountable for costs they currently externalize.

Failure mode: Voluntary payment reform. The empirical record is unambiguous: voluntary ACO participation from 2010 to 2025 produced an ecosystem in which health systems with favorable patient populations participated while health systems most responsible for high costs declined.

Vermont evidence: OneCare Vermont operated as a voluntary ACO for nearly a decade. UVMHC — Vermont's largest hospital, with approximately 50% of market share — declined to participate in OneCare's total cost of care accountability structure. Act 68 (2025) mandates global budgets covering all fourteen Vermont hospitals.

Dependency 3: Policy → Clinical (Enables)

Mechanism: Clinical transformation at scale requires statutory authorization and program funding that exists only through policy action. Certificate of Need reform, scope-of-practice legislation, telehealth parity mandates, and CCBHC certification authority are all Policy pillar products that enable Clinical pillar programs.

Failure mode: Clinical programs remain pilots. Without statutory authority requiring statewide clinical redesign, clinical innovation stays contained in institutions that choose to pursue it voluntarily.

Vermont evidence: Vermont's fourteen hospital transformation plans — 148 operational commitments submitted in January 2026 — exist because Act 167 requires them. The CCBHC expansion is driven by federal certification authority accessed through state policy action.

Dependency 4: Policy → Operations (Enables)

Mechanism: The Policy pillar's statutory mandates define what the Operations pillar must execute. Without specific statutory timelines, accountability structures, and reporting requirements, the Operations pillar lacks the urgency and specificity required for serious transformation implementation.

Failure mode: The document graveyard. Without statutory mandate, transformation plans are produced on advisory timelines with advisory accountability — carefully produced, widely circulated, and never implemented.

Vermont evidence: The AHS monthly transformation reports — required under Act 167 — are a direct product of the statutory reporting mandate. Without that mandate, AHS transformation activities would be reported at the agency's discretion, on a timeline the agency controls.

III. Technology Pillar — Outbound Dependencies (3 Relationships)

The Technology pillar has three outbound dependencies, all in the 'requires' category: Economics, Clinical, and Equity each require specific Technology pillar outputs to perform their management functions. The Technology → Economics dependency is the most consequential in the entire framework.

Dependency 5: Technology → Economics (Requires — Critical Path)

Mechanism: Value-based payment models require real-time or near-real-time data on patient attribution, utilization patterns, cost trajectories, quality metrics, and care gaps to be managed operationally. Without this data substrate, the payment model exists as a financial accountability structure with no operational management capability.

“Managing a global budget without integrated clinical and claims data is not bold reform. It is financial accountability without management capability — the conditions under which organizations can only respond to failures that have already occurred, not prevent them.” — The Health Reform Portfolio

Failure mode: The 'managing blind' failure mode. OneCare Vermont operated with 60-to-90-day-old claims data as its primary performance signal. Care managers were identifying patients who had already been hospitalized. The cost had been incurred. The intervention opportunity had passed.

Vermont evidence: The AHS-GMCB analytics vendor procurement (McKinsey, early 2026) represents Vermont's recognition that the AHEAD Model's global budgets — launching January 2027 for Medicare — cannot be managed without integrated analytics. Vermont's most significant current sequencing risk: the analytics platform must be operational before the global budget goes live, not after.

Dependency 6: Technology → Clinical (Requires)

Mechanism: Clinical population health management requires data infrastructure that makes population-level visibility possible. Without the Technology pillar's analytics and HIE capabilities, clinical programs serve the patients who present for care rather than the patients the population health model identifies as highest-risk.

Failure mode: Clinical programs that work for engaged patients and miss the population. Care management programs without risk stratification data identify patients through provider referral rather than through predictive modeling that identifies patients approaching crisis.

Vermont evidence: Vermont's Blueprint for Health community health teams operate with panel data from the Blueprint's registry system. The Blueprint's documented 5.8:1 ROI reflects the advantage of data-enabled care management. The next generation of clinical programs requires the analytics vendor's population health platform to achieve a comparable data advantage at system scale.

Dependency 7: Technology → Equity (Requires)

Mechanism: Equity measurement requires data stratified by race, ethnicity, income, geography, language, and disability status. Without this stratification capability, equity analysis is limited to aggregate outcomes that conceal disparities.

Failure mode: Average outcome improvement masking disparity widening. Equity goals are embedded in statutory language but cannot be measured with the available data.

Vermont evidence: Vermont's VHCURES all-payer claims database has documented gaps in race and ethnicity data capture that limit the demographic stratification required for equity accountability under Acts 167 and 68. Vermont's HSA-level geographic equity monitoring depends on the analytics platform's ability to produce HSA-stratified outcome data.

IV. Economics Pillar — Outbound Dependencies (3 Relationships)

The Economics pillar's outbound dependencies run to Clinical, Equity, and Operations. All three are 'requires' relationships: the payment architecture must be in place before clinical programs can achieve their full population health function, before

equity measurement can assess whether financial incentives are producing equitable results, and before operational redesign can be calibrated to the payment environment it will work within.

Dependency 8: Economics → Clinical (Requires)

Mechanism: Clinical redesign for population health management is financially rational only when the payment model rewards these activities. Under fee-for-service, the primary care visit that prevents a hospitalization generates no revenue from the prevention.

Failure mode: Clinical redesign that reverts. Hospitals and practices that invest in prevention and care coordination find that the investment produces population health improvement — and revenue reduction. Programs are discontinued when grant funding expires.

Vermont evidence: Vermont's Blueprint for Health has sustained itself because Blueprint per-member per-month payments create a partial financial return on prevention investment. The AHEAD Model's primary care payment is designed to complete the financial alignment that Blueprint partially achieved.

Dependency 9: Economics → Equity (Requires)

Mechanism: The Economics pillar's payment design determines whether financial incentives systematically disadvantage safety-net providers serving populations with high social determinants of health burden. Global budgets set without social risk adjustment hold safety-net providers accountable for costs driven by their population's social conditions rather than their operational efficiency.

Failure mode: Payment design that penalizes serving disadvantaged populations. Safety-net providers appear to be inefficient when they are in fact serving populations whose social conditions generate systematically higher healthcare utilization.

Vermont evidence: The Northeast Kingdom — Vermont's most economically disadvantaged region, with an uninsurance rate of 6-8% against a 3% statewide average — is served by critical-access hospitals whose cost structures reflect their population's high social risk. Vermont's AHEAD State Agreement includes social risk adjustment as a requirement.

Dependency 10: Economics → Operations (Requires)

Mechanism: Operational redesign must be calibrated to the payment environment it is executing within. The administrative operations optimized for fee-for-service billing are different from the administrative operations optimized for global budget management.

Failure mode: Operations optimized for the wrong payment model. Hospitals that invest in administrative transformation before their payment model is defined build infrastructure for a system that may be substantially redesigned before the infrastructure is fully deployed.

Vermont evidence: Vermont's critical-access hospitals spend \$2,730 per adjusted discharge on administration against a \$1,427 national benchmark. The RHRC's administrative simplification work must be designed around the payment model that will be live when the shared services are operational.

V. Clinical Pillar — Outbound Dependencies (2 Relationships)

Dependency 11: Clinical → Equity (Requires)

Mechanism: Equity measurement at Stage 5 requires that clinical programs have been deployed at sufficient scale and duration to produce measurable population health signals stratified by race, income, geography, and other equity dimensions.

Failure mode: Equity measurement of programs too nascent to produce measurable signals. Equity framework structurally sound but analytically empty.

Vermont evidence: Vermont's CCBHC expansion — seven certified entities by mid-2026 — is the Clinical pillar investment whose equity implications Stage 5 will measure. These questions require the CCBHC programs to be operational and the analytics platform to be producing stratified utilization data.

Dependency 12: Clinical → Operations (Requires)

Mechanism: Clinical program implementation at scale requires operational infrastructure: trained workforce, referral pathways, documentation systems, quality measurement processes. The Operations pillar's execution function cannot be fully designed until the Clinical pillar has specified what programs must be executed.

Failure mode: Operational infrastructure built for programs that are subsequently redesigned, or clinical programs specified without operational feasibility assessment.

Vermont evidence: The RHRC's technical assistance methodology is organized around translating clinical transformation commitments into operational implementation plans. Clinical commitment precedes operational design — the correct sequencing.

VI. Equity Pillar — Outbound Dependencies (3 Relationships)

The Equity pillar's three outbound dependencies are all 'constrains' relationships — the only constraining dependencies in the framework. Payment architecture, clinical program deployment, and operational prioritization must all be designed from the outset with equity requirements built in. Retrofitting equity after these systems are live is expensive, technically difficult, and politically contentious.

Dependency 13: Equity → Economics (Constrains)

Mechanism: The Equity pillar imposes social risk adjustment requirements on the Economics pillar's global budget methodology. Without social risk adjustment, global budgets systematically disadvantage hospitals serving populations with high SDOH burden whose healthcare utilization is driven by social conditions the hospital cannot control. The constraint must be embedded in the payment design process before the budget methodology is finalized.

Failure mode: Global budgets that redistribute financial pressure toward safety-net providers. Hospitals serving disadvantaged populations are penalized for their population's social risk, not their operational inefficiency.

Vermont evidence: Vermont's AHEAD State Agreement requires social risk adjustment in the AHEAD global budget methodology. The specific methodology — what social risk variables are included, how they are weighted — is the most consequential technical design choice for the equity implications of the payment model.

Dependency 14: Equity → Clinical (Constrains)

Mechanism: The Equity pillar imposes geographic and demographic reach requirements on Clinical pillar program deployment. Clinical programs must be designed and sited to reach underserved communities, not merely available in communities that already have adequate healthcare infrastructure.

Failure mode: Clinical transformation that serves the well-served. New clinical programs are deployed in communities with existing healthcare infrastructure. Rural communities, communities of color, and low-income populations receive less investment.

Vermont evidence: Vermont's Northeast Kingdom — Caledonia, Essex, and Orleans counties — faces the state's most severe combination of healthcare access deficit and social risk. Act 68's geographic equity monitoring requirement at the HSA level is the policy mechanism that makes this constraint operational.

Dependency 15: Equity → Operations (Constrains)

Mechanism: The Equity pillar imposes prioritization requirements on the Operations pillar's implementation sequencing. When organizational capacity is limited, equity constraints require that underserved communities receive prioritized attention. Operational prioritization driven by administrative convenience rather than equity need produces a transformation that reaches communities in order of their existing organizational capacity — the inverse of equity priority.

Failure mode: Operational sequencing that follows organizational convenience. Technical assistance is provided first to hospitals with existing transformation capacity. Critical-access hospitals in underserved communities receive assistance last.

Vermont evidence: Vermont's RHRC engagement mandate explicitly prioritizes rural and critical-access hospitals.

Vermont's proposed HSA Coordinator positions — AHS staff assigned to provide operational support at the hospital service area level — are designed to operationalize the equity constraint on operations.

VII. The Master Dependency Table

The following table consolidates all fifteen directed dependency relationships, their types, mechanisms, and failure modes. It is designed as the analytical reference for the execution sequence developed in Chapter 2 and for the risk assessment any reform architect must conduct before making investment and sequencing decisions.

From	To	Type	Mechanism	Failure Mode	Vermont Evidence
Policy	Technology	Enables(Critical Path)	Policy secures the transformation capital — federal grants and agreements — that funds the statewide Technology build at a scale no individual institution can finance independently.	Fragmented technology investments producing disconnected data silos. No statewide interoperability. Population health management remains analytically impossible.	\$195M RHT award funds AI scribe, RPM, broadband, and CIN — all enabled by the AHEAD State Agreement and Act 167 policy work.
Policy	Economics	Enables(Critical Path)	Mandatory authority converts voluntary payment model participation into binding obligation, preventing the highest-cost actors from opting out of accountability.	Voluntary ACO ecosystem. High-cost actors decline. Results limited to willing participants with favorable populations. No structural cost reduction.	OneCare: voluntary decade, UVMHC not participating. Act 68: mandatory global budgets, all 14 hospitals covered under GMCB authority.
Policy	Clinical	Enables	Statutory authorization, program funding, and statewide planning requirements mandate clinical transformation at population scale rather than individual institution choice.	Clinical innovation stays in willing institutions. No population-level coverage. Programs remain pilots indefinitely.	148 hospital plan commitments exist because Act 167 requires them. Blueprint near-universal coverage driven by GMCB budget conditions.

Policy	Operations	Enables	Statutory timelines and accountability structures create the urgency and specificity required for serious transformation implementation at an organizational level.	Document graveyard. Plans produced on advisory timelines with no binding accountability. Widely circulated, never implemented.	Act 167 monthly reporting mandate converts voluntary progress tracking into public accountability structure.
Technology	Economics	Requires(Critical Path)	VBC contracts and global budgets require real-time attribution, utilization, TCOC, and quality data to be managed operationally. Without this substrate, financial accountability exists without management capability.	'Managing blind.' Care managers working from 60-90 day old data. Interventions arrive after costs are incurred. Performance management is retrospective, not prospective.	OneCare's canonical failure. AHS-GMCB analytics vendor procurement required before AHEAD January 2027 global budget launch.
Technology	Clinical	Requires	Clinical population health management requires risk stratification, care gap identification, and care management targeting data that only integrated data infrastructure can produce at population scale.	Clinical programs serve presenting patients, not population priorities. High-risk patients identified after crisis presentation rather than before.	Blueprint registry data enables 5.8:1 ROI. AHEAD-scale clinical programs require analytics vendor platform for comparable advantage at system level.
Technology	Equity	Requires	Equity measurement requires data stratified by race, ethnicity, income, geography, and disability. Without stratification capability, equity analysis is limited to aggregates that conceal disparities.	Average outcome improvement masking disparity widening. Equity goals embedded in statute but unmeasurable with available data.	VHCURES race/ethnicity data gaps limit demographic stratification. HSA-level geographic monitoring under Act 68 requires analytics platform VHCURES cannot currently produce in real time.
Economics	Clinical	Requires	Clinical redesign for population health is financially rational only when the payment model rewards prevention and care coordination. Under FFS, these activities reduce revenue.	Clinical redesign that reverts. Programs prove effective in funded pilots. Financial return cannot be captured under existing payment model. Programs discontinued when grant funding expires.	Blueprint PMPM creates partial alignment. AHEAD primary care payment completes it. Until AHEAD is live, clinical investment operates on

					incomplete incentive alignment.
Economics	Equity	Requires	Equity pillar's Stage 5 assessment of financial incentive equity requires the payment model to be live and producing measurable financial signals for assessment.	Equity measurement of a payment model not yet deployed. Equity goals aspirational because the economic intervention they assess has not yet occurred.	AHEAD global budgets must be live before equity assessment of their distributional effects is possible. Social risk adjustment audit requires post-launch data.
Economics	Operations	Requires	Operational redesign must be calibrated to the payment environment it will execute within. FFS operations and global budget operations are structurally different functions.	Operations optimized for the wrong payment model. Shared services built for a payment architecture that is subsequently redesigned.	\$1,303/discharge administrative gap. RHRC shared services design must target the global budget operational model, not a hybrid FFS-to-global budget transitional model.
Clinical	Equity	Requires	Equity measurement of clinical program reach requires programs to be operational at sufficient scale and duration to produce measurable stratified population health signals.	Equity measurement of programs too nascent to produce measurable signals. Equity framework structurally sound but analytically empty.	CCBHC geographic reach equity assessment requires seven-entity deployment to be operational and analytics platform producing stratified utilization data.
Clinical	Operations	Requires	Operational infrastructure for clinical programs must follow program specification. Clinical design defines what operations must execute; operational planning follows clinical commitment.	Operational infrastructure built for programs subsequently redesigned, or programs specified without operational feasibility assessment.	RHRC translates 148 clinical hospital plan commitments into implementation plans. Clinical commitment precedes operational design — the correct sequencing.

Equity	Economics	Constraints	Equity imposes social risk adjustment requirements on global budget methodology from design outset. Must be embedded before methodology is finalized.	Global budgets penalizing safety-net providers for serving high-SDOH populations. Financial pressure on exactly the hospitals serving the most vulnerable communities.	AHEAD State Agreement requires social risk adjustment. GMCB methodology development is the critical design choice for NE Kingdom hospital financial viability.
Equity	Clinical	Constraints	Equity imposes geographic and demographic reach requirements on clinical program siting and design from the outset, not as an afterthought.	Clinical transformation serving the well-served. Programs deployed in communities with existing infrastructure. Underserved communities receive less investment.	NE Kingdom priority for CCBHC siting, AHEAD primary care targeting, and REH model deployment must be embedded in clinical program design, not assessed after deployment.
Equity	Operations	Constraints	Equity imposes prioritization requirements on operational sequencing. Underserved communities must receive prioritized technical assistance and organizational support, not deferred attention.	Operational sequencing following organizational convenience. Hospitals with existing capacity receive support first. Equity gap widens during transformation before narrowing.	RHRC mandate prioritizes rural and CAH hospitals. HSA Coordinator positions embed state capacity in highest-need communities.

VIII. Failure Cascade Analysis

The dependency architecture generates specific, testable predictions about what fails when any pillar is absent, underfunded, or activated in the wrong sequence.

Pillar Absent or Severely Underdeveloped	First-Order Failure	Second-Order Cascade
Policy	Voluntary reform. High-cost actors opt out. No mandatory architecture for accountability.	Without mandatory Economics, technology investments serve willing participants only. Clinical programs remain pilots. Operations lacks statutory urgency.
Technology	'Managing blind.' Global budgets and VBC contracts exist without operational management capability.	Economics produces unexpected financial losses. Clinical programs cannot risk-stratify. Equity measurement is analytically impossible.

		Operations cannot measure implementation effectiveness.
Economics	Clinical redesign without aligned incentives. Prevention investment is financially penalized.	Clinical programs prove effective in pilots but cannot achieve financial sustainability or system-wide scale. Operations builds infrastructure for programs that cannot sustain themselves.
Clinical	Financial pressure without clinical pathway. Global budgets create incentive to reduce costs with no mechanism to do so through improved population health.	Economics produces access restriction rather than population health improvement. Equity worsens as restricted access falls hardest on disadvantaged populations.
Equity	Transformation that improves averages while widening disparities. Success metrics show improvement while disparate harm persists.	Payment models penalize safety-net providers. Clinical programs concentrate in well-served communities. Political legitimacy of transformation collapses when disparity widening becomes visible.
Operations	The document graveyard. Technically correct reforms that cannot be implemented at scale. Strategy without execution.	Policy mandates produce legal exposure without operational compliance. Technology platforms are procured and unused. Clinical programs are specified but not implemented.

Sources: six-pillar framework documentation; Acts 167 and 68 of 2025; Oliver Wyman Act 167 Community Engagement: Recommendations (August 2024); Vermont AHE.AD State Agreement (January 2025); Vermont RHT Program Application (November 2025); GMCB records; OneCare Vermont public filings; CMMI model evaluation (2023); AHS Hospital Transformation Plans Analysis (January 2026); HTR Analysis (2026).

The Execution Sequence

Why Order Is Not Optional — The Dependency Logic That Determines When Each Pillar Goes Live

Chapter 2 derives the six-stage execution sequence from the dependency architecture established in Chapter 1A. It uses OneCare Vermont's failure as the primary negative proof case and Vermont's Blueprint for Health as the primary positive proof case. It resolves the Technology-Economics dependency in detail, distinguishing Economics-as-design from Economics-as-management and Policy-as-capital from the incentive architecture. Each stage is developed with its rationale, its Vermont anchor, and its failure mode.

I. The Question Every Reform Architect Must Answer First

The six pillars of healthcare transformation are often presented as simultaneous imperatives. Each is necessary. Each depends on the others. Remove any one and the system fails. All of this is true. But it obscures the single most consequential practical question that every health system leader, state official, and reform architect must answer before a single dollar is spent or a single care model is redesigned: in what order do you build this?

The answer is not arbitrary. It is not the order in which the pillars are typically discussed, funded, or politically convenient. The execution sequence is determined by dependency logic — the hard reality that some things cannot function until other things are already in place. Getting the order wrong does not merely slow progress; it produces structural failures that are expensive to reverse and politically difficult to explain.

Vermont provides both the positive and negative proof cases for this argument. The Blueprint for Health — operational for nearly two decades, delivering a documented 5.8 to 1 return on investment — demonstrates what is achievable when foundational stages are built in the right order. OneCare Vermont demonstrates with equal clarity what happens when foundational stages are skipped or assumed.

Vermont: The Crisis That Made Sequencing Non-Negotiable
The sequencing argument that follows is grounded in Vermont's fiscal reality. These numbers explain not just why Vermont needed reform — but why the sequencing of that reform could not be improvised.
Nine of Vermont's fourteen hospitals reported operating losses in FY2023, with deficits reaching -8.9%. Oliver Wyman projects thirteen of fourteen in losses by 2028 absent structural intervention. The cumulative five-year system deficit is projected between \$700 million and \$2.4 billion.
Vermont's average silver exchange plan premium rose from \$456 per month in 2018 to \$948 in 2024 — a 108% increase — while median household income grew 22%. UVMHC, with approximately 50% of the state's hospital market share, charged approximately 417% of Medicare rates for outpatient services in 2022.
Vermont's working-age population is projected to decline 13% by 2040 while residents aged 65+ rise from 21.7% to over 30%. This demographic shift progressively transfers patients from commercial insurance to Medicare and Medicaid, both of which reimburse hospitals below cost, collapsing the cross-subsidy that has sustained Vermont's rural hospital system.
Vermont did not choose careful sequencing because it was disciplined. It was forced to sequence correctly because it had run out of time to get it wrong.

II. The OneCare Failure: A Sequencing Autopsy

OneCare Vermont's failure was not random and it was not primarily the result of bad intentions or poor execution. OneCare failed because of sequencing errors baked into its design from the beginning. Identifying those errors precisely is more useful than simply noting that the ACO wound down after a decade of operation.

An Accountable Care Organization is a group of hospitals, physicians, and other providers who coordinate care for a defined patient population with the goal of reducing costs. Vermont created OneCare as its primary ACO vehicle under the Total Cost of Care model — a payment structure that held OneCare financially responsible for the total healthcare spending of an attributed Vermont population. On paper, this was sound design. In practice, it encountered three sequencing failures that compounded into a cascade the organization could not survive.

Sequencing Failure 1 — Economics Without Policy Readiness

The most fundamental problem with OneCare was that Vermont attempted to operate a sophisticated payment reform model requiring system-wide accountability without the policy foundation to enforce participation. Provider participation in OneCare was voluntary. Hospitals, physician practices, and other providers could choose whether or not to join.

This matters structurally. The Total Cost of Care model holds the ACO financially accountable for the total spending of an attributed population. But when high-cost, high-revenue providers remain outside the ACO because participation is optional, the ACO is held responsible for costs it cannot influence. A hospital system generating significant revenue from inpatient admissions has a rational financial incentive to stay outside a model that rewards reducing those admissions.

What Optional Participation Actually Means for a Hospital CFO

Consider a chief financial officer of a Vermont hospital generating \$40 million annually from inpatient admissions. OneCare Vermont is asking the hospital to join an ACO that will hold it financially accountable for reducing those admissions — reducing its revenue. Participation is voluntary.

Three choices are available: join and accept the risk; join and manage costs without affecting the revenue base; or decline to join. For many hospital systems operating at or near breakeven in Vermont's financially stressed environment, the rational choice was the third. This is not a failure of values. It is a predictable response to an incentive structure that the policy framework failed to close.

Vermont's legislative progression from Act 167 through Act 51 to Act 68 reflects the hard-won recognition of this failure. Mandatory architecture converts voluntary reform into binding structural change.

Sequencing Failure 2 — Economics Without Technology: The 'Managing Blind' Failure Mode

OneCare attempted to manage a complex, population-level payment model involving real financial risk without the technology infrastructure required to do so. OneCare did not build or acquire an integrated data system combining timely clinical data with claims data in a way that made population health management operationally feasible.

Claims data is the financial record of healthcare: every bill submitted to an insurer. Clinical data is the medical record: diagnoses, lab results, medications, care plans. Neither dataset is sufficient alone. Both are required, integrated, and available in near real time. OneCare operated without this integrated infrastructure at meaningful scale. The consequences were severe and entirely predictable. Care managers were working from claims data arriving 60 to 90 days after care was delivered — identifying patients who had already been hospitalized rather than preventing hospitalization.

“Managing a global budget without integrated, timely clinical and claims data is not bold reform. It is financial accountability without management capability — the conditions under which organizations can only respond to failures that have already occurred.” — *The Health Reform Portfolio*

Why Timely Data Matters as Much as Integrated Data

Consider a care manager responsible for 500 high-risk patients under a Total Cost of Care model. Her job is to identify patients who are deteriorating and intervene before they are hospitalized.

If her data arrives 60 days after the fact, she learns in March that a patient was hospitalized in January. The intervention opportunity is gone. The cost has already been incurred. She is reviewing history, not managing risk.

If her data arrives within 24 to 48 hours — integrated clinical and claims feeds updated daily — she can see that a patient filled a new prescription last week, had an abnormal lab result three days ago, and has not seen their primary care provider in six months. She can call today, before the hospitalization happens.

Vermont's analytics vendor procurement addresses this directly: VHCURES provides longitudinal population data; VITL/VHIE provides the clinical data exchange; and the AHS-GMCB Analytics Vendor provides the modeling layer for global budget management and equity monitoring. This is the substrate OneCare never built.

Sequencing Failure 3 — The Failure Cascade

The two sequencing failures above compounded into a cascade that progressively undermined OneCare's ability to function. Because participation was optional, OneCare could not achieve the provider network coverage required to manage total cost of care. Because it could not manage total cost of care, it could not demonstrate savings. Because it could not demonstrate savings, it could not attract the capital required to build the technology infrastructure that might have allowed it to manage costs more effectively. Because it lacked that technology, its care management remained limited, its financial reporting remained inadequate, and its clinical value remained undemonstrated. And because it could not demonstrate clinical value, skeptical providers had no reason to join — returning the cascade to its starting point.

This is what a sequencing failure looks like at the system level: not a single catastrophic event, but a slow structural unraveling in which each missing foundation makes the next stage harder to execute. OneCare Vermont wound down its ACO operations after roughly a decade. The failure was architectural, not operational.

Sequencing Gap	Structural Error	Direct Consequence	Cascade Effect
Policy (Stage 1 absent)	Voluntary participation — no mandate to require high-cost provider enrollment.	High-revenue hospitals opt out; ACO held accountable for costs it cannot influence.	Total cost of care cannot be managed at system level → savings targets missed.
Technology (Stage 2 absent)	No integrated real-time clinical + claims data; 60-90 day reporting lag.	'Managing blind' failure mode — care managers identify hospitalised patients after the fact, not before.	Proactive intervention impossible → high-cost events not prevented → financial exposure accelerates.
Economics (Stage 3 premature)	Payment model assumed financial risk before technology was ready to support it.	Unexpected losses → inability to demonstrate savings to payers or funders.	Capital unavailable to build technology → technology gap persists → cycle repeats.
System-level cascade	Three gaps mutually reinforcing.	Organisational wind-down after approximately 10 years of operation.	Failure was architectural — no management decision could reverse it once the sequence was locked in.

III. Resolving the Technology-Economics Dependency

The OneCare failure resolves the apparent chicken-and-egg relationship between Technology and Economics. The resolution requires distinguishing three functions that reform frameworks typically conflate:

- Economics-as-design — the analytical work of defining the payment architecture: units of accountability, performance metrics, risk allocation. This planning work can and should happen during the technology build. It does not require the technology to be running. A technology vendor building a care management platform needs to know what the payment model is measuring — the design specifies the build requirements.

- Policy-as-capital — the funding of the technology build itself. The capital for Stage 2 technology infrastructure is a Policy-pillar function, not an Economics-pillar function. Vermont's \$195 million Rural Health Transformation award is the clearest example — federal grant capital secured through the Policy pillar that funds the technology substrate. This is fundamentally different from the incentive architecture the Economics pillar creates.
- Economics-as-management — the ongoing operational work of running global budgets: monitoring performance in real time, calculating shared savings, adjusting payments, holding providers accountable. This work absolutely requires the technology infrastructure to already be fully functional.

The sequencing logic that follows is unambiguous: Policy secures both the mandate and the capital. Technology gets built — funded by Policy capital, specification driven by Economics-as-design work happening in parallel. Economics goes live as management only when the technology can support it. OneCare inverted this sequence by going live with financial risk before the technology was ready.

There is also a critical distinction between Incentive Architecture and Transformation Capital. Global budgets and reference-based pricing permanently restructure the payment environment — this is the incentive architecture that makes population health management financially rational for the long term. Transformation Capital — the RHT award, the AHEAD EAST Fund providing up to \$150 million annually — is time-limited bridge funding for infrastructure. Treating one-time grants as permanent subsidies produces reforms that are structurally misaligned from their first day of operation.

IV. The Six Stages of Execution

Stage 1: Policy — Mandatory Architecture and Transformation Capital

Policy is the absolute precondition for everything that follows. The Policy pillar has two functions no other pillar can perform: it creates the mandatory architecture that prevents high-cost actors from opting out of reform, and it secures the transformation capital that funds the technology build.

Vermont's legislative progression illustrates both functions. Act 167 established the diagnostic foundation. Act 51 defined the transformation architecture. Act 68 operationalized the mandate — converting voluntary reform into binding statutory requirements and establishing the regulatory framework for global budgets and reference-based pricing. The AHEAD Model federal agreement layered in the federal partnership and funding commitments that made the technology build financially feasible.

The \$195 million Rural Health Transformation award is a Policy-pillar outcome, not an Economics-pillar outcome. Reform architects who expect the payment model to self-fund the technology build will consistently find themselves in the same position as OneCare — financially exposed before the infrastructure is ready. Policy at Stage 1 must address the participation question directly. A reform architecture that allows high-cost providers to opt out of accountability creates a structural gap that no subsequent stage can fill.

Policy Pillar — Stage 1 Checklist	Vermont Status (April 2026)
Mandatory participation authority — can high-cost actors be required to participate?	Yes — GMCB mandatory rate-setting authority; Act 68 mandatory global budgets for all 14 hospitals
Transformation capital secured — is federal/state funding in place for the technology build?	Yes — \$195M RHT Program award (Dec 2025); AHEAD State Agreement (Jan 2025)
Statutory accountability timelines — are deadlines binding, not advisory?	Yes — Act 167 monthly reporting; Act 68 statewide strategic plan by Dec 2028
Regulatory enforcement authority — does a body exist to enforce payment mandates?	Yes — GMCB; statutory authority since FY2013 hospital budget orders

Federal relationships operational — are federal agreements signed and capital flowing?	Yes — AHEAD Cohort 2 agreement live; RHT award active; EAST Fund operational Jan 2026
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Stage 2: Technology — Building the Substrate Before Assuming Risk

Technology is the operating system on which every subsequent stage runs. This is the sequencing insight that OneCare's failure makes most vividly clear: Technology is not a layer added on top of a functioning reform. It is the foundation that makes reform functional.

The technology infrastructure required has three non-negotiable components. First, it must integrate clinical data with claims data. VHCURES without clinical data tells you what things cost but not how to change them; clinical data without VHCURES tells you about individual patients but not population-level cost patterns. Second, the data must be timely. Data arriving 60 to 90 days after care supports retrospective reporting but not proactive care management. Third, shared technology infrastructure must be available to rural hospitals that individually lack the scale to build these functions — which is what Vermont's Clinically Integrated Network provides.

Vermont faces its most significant current sequencing risk at this stage. The AHS-GMCB Analytics Vendor was procured in early 2026. The AHEAD Model's Medicare global budgets launch in January 2027. The analytics platform must be fully operational before that launch — not during it, not after it.

Stage 3: Economics — Incentive Architecture on a Visible System

Economics is the third stage, not the second, because its management function requires the technology substrate already to be operational. Vermont's economic architecture has two structural components that work together. Reference-based pricing, targeted at or below 200% of Medicare rates, addresses the unit price problem. But price controls alone create a 'Balloon Squeeze' — if unit prices are capped without controlling volume, providers generate more volume to maintain revenue. Global hospital budgets address the volume dimension by setting a fixed total revenue envelope for each hospital, making volume expansion financially irrational. Together, these instruments break the fee-for-service trap.

The equity constraint on payment design becomes operationally urgent at Stage 3. Global budgets set without social risk adjustment can inadvertently penalize safety-net providers serving high-SDOH populations. Vermont's Northeast Kingdom is the clearest example: a global budget benchmark calibrated without accounting for the region's social risk profile will generate financial pressure that reflects social disadvantage, not operational inefficiency. Social risk adjustment must be embedded in the payment design at Stage 3, not retrofitted at Stage 5.

Stage 4: Clinical — Redesigning Care on Aligned Incentives

Clinical redesign is where the economic return on the entire portfolio is realized. The Patient-Centered Medical Home transforms primary care from acute episodic treatment into proactive, team-based population management. The Collaborative Care Model integrates behavioral health into primary care. The Certified Community Behavioral Health Clinic expands access for the highest-need populations. Vermont's Blueprint for Health generated a documented 5.8 to 1 return on investment — among the most robust ROI findings in the value-based care literature.

Clinical redesign comes fourth because clinical transformation without economic alignment reverts when the incentives revert. Without a payment model that rewards the redesigned care — already live at Stage 3 — and without the technology to identify which patients need which interventions — already built at Stage 2 — clinical transformation is isolated, unsustainable, and unable to demonstrate the population-level outcomes that justify its cost.

Stage 5: Equity — Constraining Dependency and System Calibration

Equity is fifth in execution sequence but must be understood as a constraining dependency that runs through every preceding stage. At Stage 3, equity constrains the payment design through social risk adjustment. At Stage 4, equity constrains the clinical redesign through cultural competence requirements, language access mandates, and targeted deployment in underserved areas.

What Stage 5 adds is the systematic, population-level measurement of whether equity commitments embedded in earlier stages have produced equitable outcomes — and corrective action, informed by real data from a functioning system, when

they have not. Vermont's HRSN screening at scale, the EAST Fund for social determinants, and the Rural Emergency Hospital model provide the operational mechanisms for this stage.

Stage 6: Operations — Closing the Administrative Cost Gap

Operations is the execution layer that converts all prior-stage investments into organizational reality. Vermont's administrative cost data makes the operational opportunity concrete: Vermont's critical-access and rural hospitals spend \$2,730 per discharge on administration, compared to a national benchmark of \$1,427. This \$1,303 per-discharge gap represents the primary source of projected direct savings from shared services and administrative simplification.

Operations is last in execution sequence not because it is least important — administrative cost reduction is one of the most direct levers for financial sustainability — but because operational redesign requires the technology infrastructure, payment incentives, and clinical models of the preceding stages to already be in place. The Rural Health Redesign Center methodology produces durable results only when the payment environment it is optimizing for is already operational.

V. The Sequence in Practice

The six-stage sequence is a logical structure grounded in dependency relationships, not a rigid waterfall that prohibits parallel work. Payment architecture design proceeds in parallel with the technology build. Operational planning begins before the technology is complete. Clinical pilots run before the payment model fully covers the redesigned care. What the sequence provides is a framework for understanding what happens when dependencies are violated — when a later stage assumes operational risk before an earlier stage has delivered its foundational output.

“The sequence failures that produce the most damage are the ones that appear to be minor timing decisions. OneCare did not dramatically choose to ignore technology. It made the reasonable-sounding decision to launch the payment model while technology was still being developed. That decision, compounded by the absence of mandatory participation, produced a failure cascade that no amount of subsequent operational excellence could reverse.” — *The Health Reform Portfolio*

VI. Sequencing Summary

Stage	Pillar	Key Action	Why This Sequence	Vermont Anchor
1	Policy	Establish enforceable authority, mandate participation, and secure transformation capital.	Nothing can be authorized, built, or funded without statutory permission. Policy secures the capital that funds the technology build. Mandatory architecture prevents high-cost actors from opting out.	AHEAD Model federal agreement; Acts 167, 51, 68; \$195M RHT award; GMCB mandatory authority
2	Technology	Build integrated clinical + claims data infrastructure — real-time — before financial risk is assumed.	Technology is the operating system. It must be operational before Economics goes live. Managing global budgets without integrated data is the 'managing blind' failure mode that destroyed OneCare.	VHCURES; VITL/VHIE; AHS-GMCB Analytics Vendor; Vermont CIN; AI-assisted documentation
3	Economics	Deploy global budgets and reference-based pricing onto a system	Economics-as-management requires the technology substrate to already be operational. Economics-as-design happens in	Global hospital budgets; Mandatory RBP ≤200% Medicare; AHEAD EAST

		that can already see itself.	parallel during Stage 2, but financial risk is not assumed until data infrastructure is ready.	Fund up to \$150M annually; TCOC model activation
4	Clinical	Redesign care delivery models and transform clinical practice on aligned incentives.	Clinical transformation is where ROI is realized — but only when payment incentives are live and technology can identify which patients need which interventions. Without aligned incentives, redesign reverts.	Blueprint for Health (5.8:1 ROI); PCMH transformation; Collaborative Care Model; CCBHC expansion (7 entities 2026)
5	Equity	Audit, calibrate, and correct — with social risk adjustment already embedded in payment design at Stage 3.	Equity is a constraining dependency embedded in Stages 3-4 and audited here against real population data. Without social risk adjustment, global budgets penalize safety-net providers.	HRSN screening at scale; EAST Fund for SDOH; social risk adjustment audit; Rural Emergency Hospital model
6	Operations	Close the administrative cost gap and translate strategy into implemented programs.	Operations is the execution layer. Vermont's \$1,303 per-discharge administrative gap against the national benchmark is the primary source of projected direct savings.	Rural Health Redesign Center methodology; shared administrative services; 14-hospital transformation planning; Vermont CIN shared services

Sources: Act 167 of 2022; Act 51 of 2023; Act 68 of 2025; AHEAD State Agreement Cohort 2 (January 2025); Vermont RHT Program Application (November 2025); Oliver Wyman Act 167 Community Engagement: Recommendations (August 2024); AHS Health Care System Transformation Reports (2025); GMCB records; OneCare Vermont Annual Reports (2017-2024); CMMI model evaluation, CBO (2023); Vermont Blueprint for Health Annual Report and ROI Study; AHS Hospital Transformation Plans Analysis (January 2026); HTR Analysis (2026).